

NEUROLOGY & BRAIN CENTER

DEPARTMENT OF CLINICAL NEUROPHYSIOLOGY & IMAGING

NEUROLOGICAL DIAGNOSTIC WORKUP

Report ID: NNE-2026-889012

Date: August 10, 2026

Patient Name: Piyush Kumar
Age / Sex: 22 Yrs / Male
Assessment Date: August 10, 2026

Patient ID: 24MCA20002
Referring Dr: Dr. A. Sharma, MD
Status: COMPLETE / EVALUATED

1. CLINICAL HISTORY & PRESENTING INDICATION

Patient presented for a comprehensive neurological evaluation following episodic tension-type headaches and subtle paresthesia in the distal extremities. No history of focal seizures, loss of consciousness, head trauma, or motor deficits reported. The workup was requested to evaluate for central structural lesions, cortical electrical irritability, and peripheral nerve pathway integrity.

2. COMPREHENSIVE NEUROLOGICAL EXAMINATION NOTES

I. Mental Status & Cognition

- Alertness:** Fully alert and oriented x 4.
- Language:** Fluent speech, intact naming, comprehension, & repetition.
- Memory:** Intact short/long-term memory (3/3 recall at 5 min).

II. Cranial Nerves (CN I – XII)

- CN II-III:** Visual fields full. Pupils equal, round, reactive (PERRL).
- CN III, IV, VI:** EOMI without nystagmus or diplopia.
- CN V, VII:** Facial sensation intact; symmetrical facial motor function.
- CN VIII-XII:** Hearing intact bilaterally; palate elevates symmetrically.

III. Motor System & Reflexes

- Motor Strength:** 5/5 power in all major upper/lower extremity groups.
- Bulk & Tone:** Normal muscle bulk; no rigidity, spasticity, or atrophy.
- Reflexes:** Deep tendon reflexes 2+ bilaterally (Biceps, Patellar, Achilles).
- Plantar:** Flexor response bilaterally (Negative Babinski).

IV. Sensory, Coordination & Gait

- Sensory:** Intact to light touch, pinprick, vibration, & temperature.
- Coordination:** Smooth finger-to-nose and heel-to-shin testing bilaterally.
- Romberg:** Negative (stable with eyes closed).
- Gait:** Normal casual gait, tandem walk, heel, and toe walking.

3. DIAGNOSTIC NEUROIMAGING & ELECTROPHYSIOLOGY SUMMARY

Diagnostic Test	Protocol / Scope	Key Clinical Findings	Interpretation
Brain MRI	3T Multi-planar T1, T2, FLAIR w/ & w/o contrast	Normal brain parenchyma. No intracranial mass, acute infarction, hemorrhage, or demyelinating plaque. Normal ventricular size.	NORMAL
Non-Contrast CT	Axial Head CT (1.25mm slices)	No acute intracranial bleed, extra-axial fluid collection, or midline shift. Osseous structures of the skull intact.	NORMAL
Routine EEG	20-Minute Awake/Drowsy EEG (10-20 system)	Symmetric, reactive 10 Hz posterior alpha rhythm. No epileptiform spikes, sharp waves, or focal cerebral slowing.	NORMAL
EMG / NCS			NORMAL

Diagnostic Test	Protocol / Scope	Key Clinical Findings	Interpretation
	Bilateral Motor & Sensory Nerve Conduction	Normal conduction velocities, CMAP/SNAP amplitudes, and distal latencies. No active denervation on needle EMG.	

Axial T2 Brain MRI Slice
(Intact Parenchyma & Ventricles)



Figure 1.1: Brain MRI (Axial T2) demonstrating normal brain parenchymal intensity and ventricular symmetry.

10-Second Continuous EEG Waveforms (Normal 10Hz Alpha Background Rhythm)

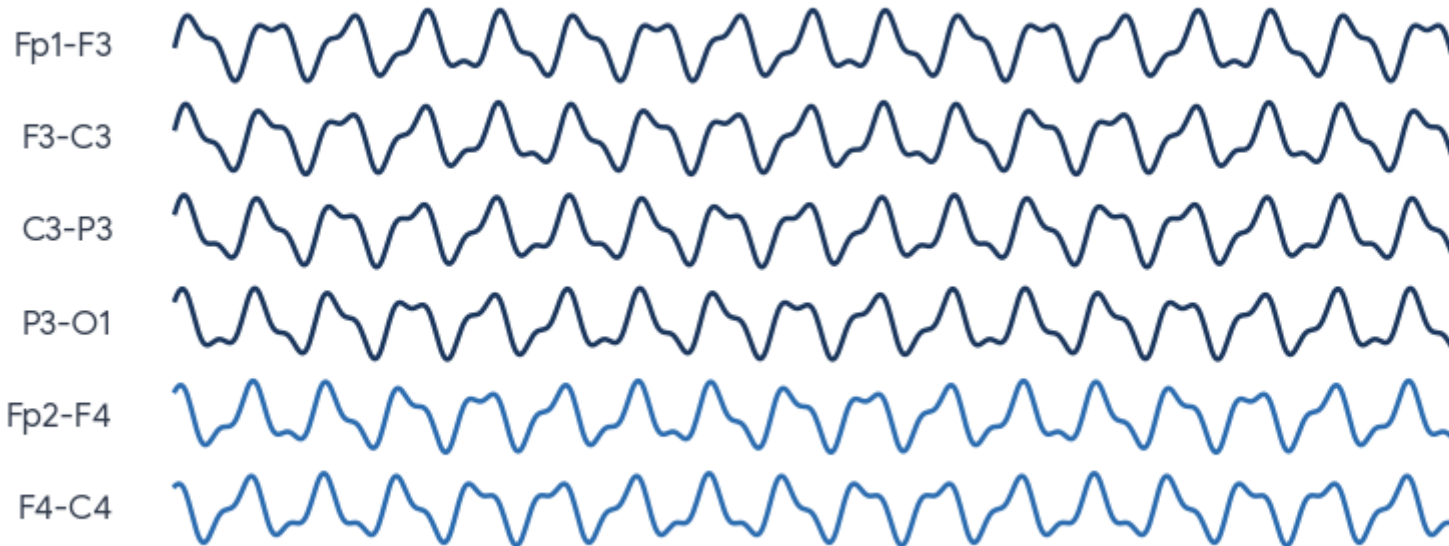


Figure 2.1: Electroencephalogram (EEG) snippet demonstrating continuous, well-organized 10Hz alpha rhythms.

4. CLINICAL IMPRESSION & DIAGNOSTIC SUMMARY

- 1. Normal Central Nervous System Structure:** MRI and CT scans reveal intact intracranial architecture without structural, vascular, or inflammatory pathology.
- 2. Normal Cortical Electrophysiology:** EEG demonstrates physiological background activity without epileptiform discharges or seizure risk.
- 3. Intact Peripheral Pathways:** EMG/NCS confirms normal distal motor/sensory nerve conduction and absence of peripheral neuropathy or radiculopathy.

5. RECOMMENDATIONS & CLINICAL PLAN

- **Symptomatic Care:** Conservative management for primary tension-type headaches (adequate hydration, stress management, ergonomic adjustments).
- **Monitoring:** Maintain a log of headache frequency and triggers if symptoms recur.
- **Follow-Up:** Routine follow-up in 6 months or sooner if new focal neurological symptoms (unilateral weakness, vision changes, or gait instability) develop.

Dr. Rajesh V. Nambiar, MD, DM

Consultant Neurologist &

Neurophysiologist

Reg. No: FMC-88412-N

* This electronic diagnostic report is validated by the attending neurophysiologist.
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